



Patient Profile

Name: Mr. Saubhik Bhaumik

Age:

Gender: Male

Summary of Results

All laboratory parameters are within their reference ranges. The predictive model flags the profile as **Normal**, indicating no evidence of significant hepatic or systemic abnormalities based on the current data set.

Detailed Test Explanations

- **Serum Bilirubin (Total)** – 0.9 mg/dl (reference 0-1.2). Total bilirubin reflects overall hepatic excretion and hemolysis; levels within range suggest normal bilirubin metabolism.
- **Serum Bilirubin (Direct)** – 0.2 mg/dl (reference 0-3). Direct (conjugated) bilirubin indicates hepatic conjugation capacity; a normal value implies adequate hepatic processing of bilirubin.
- **Serum Bilirubin (Indirect)** – 0.70 mg/dl (reference 0-1). Indirect (unconjugated) bilirubin, elevated by hemolysis or impaired conjugation; normal in this case.
- **SGPT (ALT)** – 36 U/L (reference 13-40). ALT is a hepatocellular enzyme; normal suggests no active hepatocellular injury.
- **SGOT (AST)** – 32 U/L (reference 0-37). AST is less liver-specific but remains within normal limits, supporting healthy liver function.
- **Serum Alkaline Phosphatase** – Not provided.
- **Serum Protein** – 7.2 g/dl (reference 6-8.3). Total protein reflects nutritional and hepatic status; within normal range.
- **Serum Albumin** – 4.7 g/dl (reference 3.5-5.2). Albumin indicates protein synthesis and nutritional health; normal value confirms adequate hepatic protein production.
- **Globulin** – 2.5 g/dl (reference 1.8-3.6). Globulin comprises immune globulins; within normal limits, indicating no overt immune or hepatic dysfunction.
- **A/G Ratio** – 1.88 (reference 1.1-2.4). Albumin to globulin ratio in normal range, supporting balanced protein synthesis.

Overall Interpretation

The patient's liver function panel, including bilirubin fractions, transaminases, and protein synthesis markers, demonstrates values within normal reference intervals. The absence of elevated enzymes or bilirubin indicates no current evidence of hepatic injury, cholestasis, or hemolysis. The predictive model corroborates this assessment by labeling the profile as normal, suggesting no immediate clinical concern from these laboratory findings.

Recommendations

- Continue routine annual health screening, including repeat liver function testing if clinically indicated.
- Maintain a balanced diet rich in fruits, vegetables, and adequate protein to support hepatic function.

- Avoid excessive alcohol consumption and monitor for any symptoms of liver dysfunction (jaundice, abdominal pain).
- If any new symptoms arise or if the patient engages in medications known to affect liver enzymes, consider prompt re-evaluation.

Disclaimer

This report is for informational purposes only and does not constitute medical advice. Consult a qualified healthcare professional for personalized interpretation and clinical decision-making.